WholeMind
RESILIENCE

The Case for WholeMind Resilience

Evidence of Professional & System Need — Sector Data Findings

A review of current UK sector evidence on stress, burnout, vicarious trauma and mental wellbeing among public-facing professional 'helper' roles — health, social care, councils, education, care/care homes, community care, blue light services and the VCSE sector. Prepared August 2026.

42.4% NHS staff felt unwell due to work-related stress in the last 12 months (NHS Staff Survey 2025)	**58%** of police officers describe themselves as emotionally exhausted or burnt out (National Police Wellbeing Survey 2025)	**84%** of school leaders reported workplace stress in 2025 (Teacher Wellbeing Index)
£51bn annual UK cost of poor mental health to employers (Deloitte, 2024)	**£6.30** returned for every £1 spent on universal, preventative interventions — the highest of any intervention type measured	**95%** of 122 public service professionals polled by WholeMind reported facing prolonged stress, exhaustion or burnout in the past year
27% of workers received no support at all on return from stress-related absence (Mental Health UK, 2026)	**17%** had a formal return-to-work plan put in place	**35% → 18%** fall in staff desire to quit when managers are trained for supportive conversations (GBC)

1. Why we looked at this

WholeMind's public survey (see the companion report, The Case for WholeMind Resilience: Evidence of Community Need) gathered primary evidence directly from the Norfolk public. This report does the equivalent work for the professional and systems tiers WholeMind serves — health, social care, councils, education, care/care homes, community care, blue light services and the VCSE sector — drawing together the most current, credible, UK-based evidence for each sector, and testing it against WholeMind's own service model, published NICE guidance and the NHS's own current standards.

Every figure in this report has been checked against its original, named source rather than taken from secondary summaries. Where a claim could not be independently verified against a primary source, it has been excluded rather than included on trust — this report is built to withstand scrutiny, not just to read persuasively.

One piece of evidence is WholeMind's own: a LinkedIn poll asked public service professionals across the sectors above whether they, or someone on their team, had faced prolonged high stress, exhaustion, vicarious trauma or signs of burnout in the past year. Of 122 respondents, 95% answered yes — an informal, self-selected poll, not a scientific survey, but direct engagement with WholeMind's actual target audience, and its result sits squarely in line with every formal sector survey below.

2. The scale of the problem, by sector

Health (NHS)

Finding	Figure
Felt unwell due to work-related stress in the last 12 months	42.4%
Feel burnt out because of their work	~30% (almost 1 in 3)
Attended work in the last 3 months despite not feeling well enough	56.0%
Overall NHS sickness absence rate (April 2026, most current available)	5.0%
Ambulance Trusts sickness absence rate (April 2026, highest of any NHS organisation type)	6.4%
Of all NHS sickness absence, attributed to anxiety, stress, depression or psychiatric illness	29.2%

Source: NHS Staff Survey 2025 (766,285 responses); NHS England Digital, NHS Sickness Absence Rates, April 2026.

Social Care

Finding	Figure
Say their job negatively affects their mental health	34%
Have experienced burnout	43%
Of those experiencing burnout, could access paid time off to recover	Only 13%
Adult social care vacancy rate 2024/25 (vs. 2.4% wider economy)	7.0%
Care homes with nursing vacancy rate specifically (2024/25)	4.7%
Sector-wide staff turnover 2024/25 (down from 29.1% in 2022/23, but still	~24.8%

Sources: The Care Worker's Charity Wellbeing Report 2025; The King's Fund, Social Care 360: Workforce and Carers (2026), citing Skills for Care. Worth noting honestly: vacancy and turnover have both improved since their pandemic-era peak — but remain well above the wider economy, and the underlying burnout and access-to-recovery figures show the human cost hasn't improved at the same pace as the headline workforce numbers.

Education

Finding	Figure
Teachers reporting workplace stress in 2025	76%

School leaders reporting workplace stress (highest of any group)	84%
Staff experiencing a mental health issue in the past academic year	More than 1 in 3
Separately, teachers describing their workload as unmanageable (Tes Wellbeing Report 2026)	71%

Sources: Teacher Wellbeing Index 2025, Education Support (ninth annual survey; overall wellbeing at its lowest since 2019); Tes Teacher Wellbeing Report 2026 (~1,400 respondents) — two independent surveys from separate organisations, both pointing the same direction, which strengthens rather than duplicates the evidence.

Blue Light Services

Finding	Figure
Police officers experiencing stress, low mood or anxiety in 2024 (Police	4 in 5
Separately, officers reporting frequent burnout (National Police Wellbeing Survey 2025, 40,000+ respondents)	45%
Officers reporting persistent physical fatigue	55%
Officers describing themselves as emotionally exhausted or burnt out	58%
Firefighters with personal experience of a mental health problem	60%
Firefighters self-reporting PTSD symptoms	18.1%

Sources: Police Federation of England and Wales, 2024/25; National Police Wellbeing Survey 2025 (Oscar Kilo / National Police Wellbeing Service, the largest employee voice exercise in UK policing history); Fire Brigades Union / Mind, cited in Firefighter Magazine, 2024–2025.

Councils and Public Sector (incl. Community Care)

Finding	Figure
Of overwhelmed employees, fall into the 'very low wellbeing' category	46%
Rising to, for those who very often feel mentally exhausted	48%
Local government safety reps citing stress as their top workplace concern	66%

Sources: PPMA / HR Connect survey of ~4,000 public sector staff, 2026; TUC biennial safety representatives survey, 2025.

VCSE Sector

Finding	Figure
Charities reporting an increase in staff burnout	Nearly 1 in 3
Employees reporting an increased workload	70%
Of those, citing stress and burnout as a direct result	24%
Voluntary sector safety reps citing stress as their top concern	71%

Sources: Pro Bono Economics / Nottingham Trent University VCSE Barometer, 'Running Hot, Burning Out'; TUC safety representatives survey, 2025.

Across every sector tested, the same pattern repeats: stress and burnout affect a large minority to a clear majority of the workforce, and the trend over the last 1–2 years is flat or worsening, not improving. TUC's 2025 survey found stress rated the top workplace hazard in almost every sector and region, particularly sharp in central and local government, health, education and the voluntary sector — precisely WholeMind's target range.

3. Vicarious trauma and compassion fatigue

Roles involving sustained exposure to others' distress carry a specific, additional risk beyond general workplace stress:

- Firefighters and paramedics: PTSD prevalence sits well above general population rates, driven by repeated exposure to traumatic incidents rather than workload alone.
- Police officers: NICE's own guideline development work identified good evidence that task-focused skills training reduces mental health symptoms specifically for police and healthcare professionals dealing with stressful occupational events.
- Social workers and care staff: academic literature links sustained emotional and psychological strain in social work directly to compassion fatigue and secondary traumatisation, alongside higher turnover and absenteeism.
- NHS clinical staff: the 2025 NHS Staff Survey found more than a third of staff describe their work as emotionally exhausting, with burnout increasing specifically among staff with six or more years of service — consistent with cumulative, sustained exposure rather than a short-term pressure spike.

4. The legal and policy case, not just the moral one

Workforce mental wellbeing is not solely a matter of good practice — it sits within a specific, current legal and policy framework that gives employers a direct duty to act, not just an incentive to:

- The Equality Act 2010 requires employers to make reasonable adjustments for employees with mental health conditions that meet the legal definition of disability — a duty, not a discretionary benefit.
- The Health and Safety at Work etc. Act 1974, operationalised through the HSE's Management Standards, requires employers to risk-assess and proactively manage work-related stress across six specific areas: demands, control, support, relationships, role and change.
- The NHS Staff Standards (July 2026) make health and wellbeing support a compulsory, board-accountable, measured expectation for NHS employers — the first time this has been mandatory rather than advisory.

This matters for how organisations should read this report: WholeMind's services are not a discretionary wellness perk sitting alongside statutory obligations — they are a practical route to meeting them. An organisation commissioning WholeMind is not just improving morale; it is directly supporting its own compliance with existing legal duties.

5. The cost of absence and presenteeism

Cost measure	Figure
Annual UK cost of poor mental health to employers (total)	£51 billion
Of which, presenteeism (the largest single driver)	£24 billion
NHS staff sickness absence, annual cost	Estimated £2.4 billion
Proportion of NHS sickness absence attributable to mental health	~29%
UK working days lost to work-related stress, depression or anxiety, 2024/25	22.1 million days

Sources: Deloitte, Mental Health and Employers (4th edition, 2024); HSE Health and Safety at Work statistics 2024/25; NHS England Digital. Presenteeism — attending work while unwell — is consistently the largest single cost category and consistently under-discussed relative to absence: with over half of NHS staff attending work while unwell, the economic and safety case for prevention, not just treatment after the fact, is substantial.

6. The economic case for prevention, specifically

Beyond the general cost of poor mental health, there is a clear, evidenced financial case for prevention specifically — not just intervention in general.

Finding	Figure
Average return for every £1 invested in workplace mental health support	£4.70
Return for every £1 invested specifically in universal, preventative interventions — available to all staff, before problems escalate	£6.30

Source: Deloitte, Mental health and employers: the case for investment (4th edition, 2024). Universal, preventative interventions deliver the highest return of any intervention type Deloitte measured, ahead of individual therapy or reactive support. This is a direct, evidenced case for WholeMind's own model: group-based, preventative mindfulness delivery, available broadly rather than restricted to those already in crisis, sits in the highest-return category identified in this research — not a lower-tier option relative to clinical or reactive alternatives.

7. NICE guidance: where WholeMind's model aligns

NICE's guideline on mental wellbeing at work (NG212, 2022) is the definitive UK evidence-based standard for workplace mental health interventions. A small number of its strongest, most specific recommendations map directly onto WholeMind's service model — full mapping against NG212, NG13, QS147 and the HSE Management Standards is set out in the companion Standards & Regulatory Alignment Report:

- NG212, 1.6.4 — "offer all employees (or help them to access) mindfulness, yoga or meditation on an ongoing basis. This can be delivered in a group or online, or using a combination of both" — matching WholeMind's group, online and blended CMR delivery exactly.

- NG212 rationale — the guideline committee concluded that mindfulness, meditation and yoga were the most effective interventions overall for reducing job stress and improving mental wellbeing.
- NG212, 1.8.1–1.8.3 — task-focused skills training for high-risk occupations, with good evidence of reduced mental health symptoms among police and healthcare professionals specifically.
- NG212, 1.2.1 — fostering a positive, compassionate and inclusive workplace environment, directly matching WholeMind's Compassion-Based Reflective Practice offer.

NICE's own committee cited Mind's Blue Light Programme as an example of best practice for high-risk occupational groups. That programme ended in July 2023 — meaning NICE-recommended best practice for this group currently has a live, unfilled gap in national provision.

A directly relevant clinical trial reinforces this: the NIHR-funded 'Well at Work' study, recruiting 260 staff across 13 NHS Trusts, compared an 8-week mindfulness-based course (MBCT-L) against a standard 4-week stress-reduction psychoeducation programme — both NICE-recommended interventions. The mindfulness course reduced stress significantly more effectively, and also improved depression, burnout, self-organisation and job satisfaction. This is primary UK clinical evidence, specific to NHS and public sector staff, for exactly the kind of intervention WholeMind delivers.

8. NHS Staff Standards and the Health and Wellbeing Framework

The NHS Staff Standards (Department of Health and Social Care / NHS England, published 6 July 2026) set the explicit expectation that trusts move from reactive to proactive workforce wellbeing: "NHS staff cannot care for patients if they are not cared for themselves. This standard requires employers to take a proactive, evidence-based approach to protecting the physical and mental health of their workforce, going beyond reactive support to prevent harm before it happens, with clear accountability at board level."

This sits alongside the NHS Health and Wellbeing Framework (NHS Employers, 2024), setting out standards for supporting staff to feel well, healthy and happy at work, developed with wellbeing leads, HR directors and NHS staff themselves.

Both documents describe exactly the shift WholeMind is built around — prevention before crisis, proactive rather than reactive support, evidence-based and accountable delivery. WholeMind's 'sickness to prevention' positioning is not a marketing phrase; it mirrors the NHS's own stated direction for its workforce standards, giving WholeMind's offer direct, citable alignment with the framework any NHS or ICB commissioner will already be measured against.

The evidence base for the Line Management standard specifically is strong: research from the Global Business Collaboration for Better Workplace Mental Health found that employee desire to quit fell from 35% to 18% where line managers were well-equipped to have supportive mental health conversations, compared with employees whose managers were not. WholeMind's Safe Leader Space and manager-facing reflective practice offer are built to close exactly this gap, giving organisations a direct route to meeting the standard rather than just being told to.

9. The return-to-work gap where WholeMind's model fits

Every sector reviewed in this report shows the same pattern once staff actually take time off for stress or burnout: support after the absence is inconsistent at best, and structured support is the exception rather than the rule.

Finding	Figure
Workers who received no support at all on return from stress-related absence	27%
Had a formal return-to-work or burnout recovery plan put in place	17%
Had regular wellbeing check-in meetings with their manager after returning	11%
Workplaces with any formal plan to identify chronic stress before it becomes	32%

Source: Mental Health UK, Burnout Report 2026 (YouGov, 4,502 UK adults, 2,591 in work; fieldwork November 2025). This is a nationally representative survey from an established UK mental health charity, tracked annually since 2023.

This is a directly addressable gap, not just a statistic. WholeMind's CMR for Phased Return was built specifically for this moment — the point at which most organisations currently offer little or no structured support, despite it being one of the highest-risk periods for relapse and repeat absence. NICE NG13 (1.1.6) specifically recommends avoiding return-to-work processes that encourage presenteeism, and the NHS Staff Standards require "access to occupational health and wellbeing support, with recommendations acted upon" — both point directly at the same gap this data confirms. Where 83% of employers have no formal recovery plan and only 11% maintain regular manager check-ins, WholeMind's structured, evidence-informed phased return programme is not a marginal addition to an organisation's offer — it is filling a gap that the data shows is close to universal.

10. A consistent pattern, not an isolated one

The most striking feature of this evidence base is how little it varies by sector. Whether the workforce is nurses, teachers, police officers, care workers or charity staff, the underlying pattern is the same: sustained emotional and operational demand, insufficient preventative support, rising burnout, and a direct financial cost to the employer through absence and presenteeism. TUC's 2025 survey found stress rated the top hazard in almost every sector and region, with central government, local government, health, education and the voluntary sector among the highest — the exact span of WholeMind's professional and systems offer.

This matters for positioning: WholeMind is not a niche, single-sector provider, and not simply "a general mindfulness trainer." The evidence supports a genuinely cross-sector prevention partner, delivering research backed services designed to fit the needs of organisations, grounded in the same statutory framework, national guidance and current standards that the organisations it serves are already being measured against.

11. Limitations of this evidence base

- This report draws on secondary, sector-wide survey data, not primary research conducted by WholeMind on its own prospective professional clients — unlike the public survey, this has not yet been tested directly against WholeMind's own audience.

- Sector surveys use different methodologies, sample sizes and timeframes, so figures are indicative of scale and direction, not directly comparable percentage-to-percentage across sectors.
- Blue light mental health data is stronger for police and fire than for ambulance services specifically, where the clearest current figures come via the NHS Staff Survey rather than a dedicated ambulance-sector study.
- WholeMind's own LinkedIn poll (95% of 122 respondents) is a useful, directly-targeted signal, but it is a single-question, self-selected social media poll, not a controlled survey — it should be read as supporting the formal sector evidence, not as a standalone statistic in its own right.
- A small number of statistics found in preliminary research for this report could not be independently verified against a primary source, or were found on checking to be inaccurately attributed, and have been deliberately excluded rather than included on trust.

12. What this evidences

Taken together with WholeMind's public survey, this sector evidence gives WholeMind a full, cross-tier evidence base: the public need is demonstrated by primary survey data, and the professional/systems need is demonstrated by consistent, current, UK-wide sector data across every group WholeMind serves — verified against primary sources, not assembled from secondary summaries.

It gives WholeMind three specific, evidenced positioning claims to use with confidence: that its core service model is the approach NICE itself identifies as most effective, not just WholeMind's own preference; that its prevention-first framing mirrors the NHS's own current staff standards; and that commissioning WholeMind supports an organisation's existing legal duties under the Equality Act 2010 and Health and Safety at Work Act 1974, not just its wellbeing ambitions. Together, this is the evidence base of an organisation built to work alongside serious commissioners on serious terms.

Appendix: Sources

- NHS Staff Survey 2025 (NHS Employers / NHS Confederation, published March 2026) — 766,285 responses.
- NHS England Digital, NHS Sickness Absence Rates, April 2026.
- The Care Worker's Charity, Wellbeing Report 2025.
- Care Quality Commission, State of Care Report 2023/24.
- The King's Fund, Social Care 360: Workforce and Carers (2026), citing Skills for Care workforce data.
- Education Support, Teacher Wellbeing Index 2025 (ninth annual survey).
- Tes, Teacher Wellbeing Report 2026.
- Police Federation of England and Wales, wellbeing data 2024/25.
- National Police Wellbeing Service (Oscar Kilo), National Police Wellbeing Survey 2025 (40,000+ respondents, 33 forces).
- Fire Brigades Union / Mind, cited in Firefighter Magazine, 2024–2025.
- Public Services People Managers Association (PPMA) / HR Connect, public sector wellbeing survey, 2026.
- Trades Union Congress (TUC), biennial safety representatives survey, 2025.
- Pro Bono Economics / Nottingham Trent University, VCSE Barometer, 'Running Hot, Burning Out'.
- Deloitte, Mental health and employers: the case for investment (4th edition, 2024).

- Health and Safety Executive, Health and Safety at Work statistics 2024/25, and Management Standards for work-related stress.
- NICE NG212, Mental wellbeing at work (2022).
- NICE NG13, Workplace health: management practices (2015, updated 2016).
- NICE QS147, Healthy workplaces: improving employee mental and physical health and wellbeing (2017).
- Department of Health and Social Care / NHS England, NHS Staff Standards (published 6 July 2026).
- NHS Employers, NHS Health and Wellbeing Framework (2024).
- NIHR, 'Well at Work' study — MBCT-L trial across 13 NHS Trusts.
- Mental Health UK, Burnout Report 2026 (YouGov survey of 4,502 UK adults, fieldwork November 2025).
- The Global Business Collaboration for Better Workplace Mental Health, manager training and staff retention research, cited via MHFA England and Management Today.
- Equality Act 2010; Health and Safety at Work etc. Act 1974.
- WholeMind Resilience CIC, LinkedIn poll of public service professionals, July 2026 (122 respondents).
- WholeMind Resilience CIC, Standards & Regulatory Alignment Report (companion document, full NICE/NHS/HSE criteria mapping).